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Prompt #3

Cultural Specificity of the Abnormal: The Impact of Culture on Mental Health and Feeling

According to the American Psychiatric Association (APA), the Diagnostic and Statistical Manual of Mental Disorders (DSM) helps "define and classify mental disorders [to] improve diagnoses, treatment, and research". The DSM does this by describing the "necessary" symptoms and clinical phenomena for someone to be diagnosed with a mental illness. While created by an American organization, the DSM is used around the world with the next popular alternative being the International Classification of Diseases (ICD) – classifications based on the French Bertillon Classification of Causes of Death. In "Beyond the 'New Cross-cultural Psychiatry'", Laurence Kirmayer argues that, "Efforts to develop an international nosology and standardized approaches to diagnosis and treatment remain highly biased toward the Euro-American constructs developed over the preceding century" (Kirmayer, 136). Given this, it seems problematic that the DSM and ICD are recognized to be universal authorities on diagnosis. In this essay, I will argue that conceptions of health and normality, and therefore illness and abnormality, are uniquely defined and constructed by socio-cultural contexts. Thus, accurate diagnosis and treatment of mental illness must be inextricably tied to the culturally specific practices, emotions, and context an illness arises in.

In "Anthropology and the Abnormal", Ruth Benedict claims that, "normality is culturally defined. An adult shaped to the drives and standards of either of these cultures, if he were transported

into our civilization, would fall into our categories of abnormality" (Benedict, 72). The DSM's categorization of mental disorders on a physiological, biological, and behavioral basis tells us what Americans find abnormal, but it is unfair to say that these traits and symptoms signify abnormality elsewhere in the world. As previously mentioned, Kirmayer notes that the Euro-American construct of mental health has greatly influenced diagnoses. He continues to say that, "Psychiatry itself is an agent of globalization" (Kirmayer, 131). The Euro-American conception of mental health as "biological (or, even more reductively, as genetic) is an ideological move", serves this globalization process and contributes to the success of Western pharmaceutical companies (Kirmayer, 131). The pharmaceutical industry, a product of American capitalist culture, has undoubtedly shaped American psychiatry, so it would make little sense to assume that the DSM, created by the APA, isn't uniquely tied to Western, and specifically American, conceptions of mental health. The imposition of Western conceptions of health in non-Western cultures, consequently, seems puzzling and capitalistic: "For example, a WHO-sponsored meeting in Japan…discussed the utility of the concept of somatization, a construct born of western dualism… Japanese notions of the inseparability of body and mind are then displaced by a Western tradition of dichotomous thinking at the same time that others are calling for strategies to heal this split" (Kirmayer, 136). Now that we've briefly discussed the Western view, we will turn to exploring non-Western notions of health.

Studies of the Javanese and Balinese experiences with mental illness and well-being clearly show that different definitions of normality and health exist in each culture. In Subandi's study of schizophrenia in Javenese society, we read of a common Javanese folktale, *Suminten Edan,* surrounding the disorder: "*Suminten Edan*…represents one Javanese cultural model of psychotic illness…. In terms of its therapeutic implications, first, family problems are acknowledged as a trigger for mental illness; second, the sudden onset of illness is quite striking; the patient makes a rapid recovery and quickly returns to normal life" (Subandi, 424-25). This model of mental illness

was active in the case of Tuti, a woman who developed schizophrenia as a result of tense and stressful family dynamics (Subandi, 424). Tense family dynamics seem to exist in every part of the world, so how is Tuti driven to psychosis? The Javanese ideal of "maintaining *tentrem*" is essential to understanding cases of schizophrenia in Java. In Javanese culture, *tentrem* represents a calm and peaceful state of mind that the Javanese constantly strive to have both internally and in their relationships. Disruptions to *tentrem,* like overwhelming stress or being startled, have the power in the Javanese cultural context to create psychological disturbances.

Similarly, culturally specific emotions can dictate health. Wikan's work on the Balinese elucidates the fact that health in Balinese society is directly tied to one's ability to balance their emotions – fostering *keneh* means acknowledging that to be physically healthy is to be mentally healthy (Wikan, 294). Furthermore, health becomes a collective concern, and emotion "is bound in a conception of social obligation and balance in the cosmos, experienced bodily as jeopardized when deities, demons or fellow beings are angered" (Wikan, 295-296). The importance of both individually and collectively managing the heart helps to characterize the abnormal; those who refrain from this collective effort to maintain *keneh* are deemed selfish or angry, traits associated with illness and misfortune (Wikan, 295-298). Just as the lack of *keneh* to the Balinese brings illness, the lack of the "Javanese value of *ngemong* – providing support–" was conducive to psychosis: one Javanese woman, Susi, attributed her psychosis to suddenly moving into a household where, abnormally to her, no *ngemong* existed (Subandi, 425). It may now be clear that Western conceptions of mental health do not account for specific culturally constructed emotions or illnesses in the Javanese, Balinese, or even Japanese cultures, but what about illnesses that *are* listed in the DSM?

Mental health is not only conceived of differently from culture to culture but also experienced and felt distinctly. Subandi compares the Javanese and European experience of dementia praecox, or schizophrenia: "with respect to dementia praecox, visual and auditory hallucinations were

much less prevalent among the Javanese, and their delusions were less systematized. The prognosis of Javanese patients was significantly better than that of Europeans". In fact, Subandi also mentions that rates of recovery for patients that met all criteria for having schizophrenia tended to be higher in "developing" countries, as opposed to in "developed" countries (Subandi, 411). While these patients all meet the physiological, behavioral, and supposedly biological criteria for a diagnosis, their lived experiences with schizophrenia are ultimately determined by their cultural context.

This disconnect between physiology, symptomatic display, and recovery is not unheard of; nocebo and placebo effects have long shown us that social learning and higher-level cognitive processes can drive our symptom perception and recovery regardless of physical condition. Kirmayer also discusses the social learning aspect: "On the face of it, mental health problems may reside not only in defects in the hardware of the brain but in its software (learning) and outside the individual in interpersonal interactions in families and communities, as well as in larger social configurations" (Kirmayer, 131). In Java, social support, practicing *ngemong*, is an instrumental part of the comparatively rapid recovery of schizophrenics; conversely, as previously mentioned, a lack of *ngemong* can help induce psychosis. For the Balinese, feeling anger, or being unfriendly, is seen as a "grave threat to 'public' health" - communication and interaction in the Balinese cultural context has the power to endanger one's health (Wikan, 303). While feeling angry in Western culture may feel completely disconnected from physical illness, the importance of balance of body and soul in Balinese philosophy makes anger a biological threat in Bali; likewise, the more individualist ideals in Western cultures may contribute to the lower recovery rates for schizophrenia when compared with the high Javanese recovery rates that are arguably due to the cultural focus on support, a lack of which is a common inducer of psychosis in Java.

Given that both the conceptions and experience of mental illness are uniquely culturally constructed, treatment cannot be universal. It seems inconsistent and illogical for such a culturally

specific construction to be treated universally. Benedict helpfully claims that "we are handicapped so long as we identify our local normalities with the universal sanities" (Benedict, 76). It seems that the opposite, identifying our local abnormalities with universal insanities, is also true given the discussion thus far. To treat an imbalance, presenting as some illness listed in the DSM or ICD, in the Balinese feeling-mind with Western medicine would be irrational considering the fact that Western medicine was and is not created to treat the Balinese feeling-mind. Instead, Balinese "laughter works in ways it would not do with us…. to enhance bodily well-being" (Wikan, 308). Americans would not jump to use laughter as a treatment for illness because our culture has defined illness in the context of medicine, hospital visits, and a pause in everyday life. While *ngemong*, laughter, and *tentrem* are constantly upheld in society to avoid sickness in the community, illness is more individual and accepted as a state of being, in the West. In other words, being ill does not equate to being selfish, uncooperative, or morally compromised.

Overall, although they may manifest similarly, culture to culture, mental health issues can only truly be understood within the specific cultural context of the person feeling them. Medicine is a powerful tool that arguably works for American culture but, as Kirmayer stated, "Understanding scientific theory and its technological expressions as culturally embedded opens a space for moral and political critique" (Kirmayer, 131). He goes on to say that this recognition is essential if we want psychiatry, and its technological expressions (medicine), to help people rather than serve as a mechanism for globalization or social control. Therefore, we should critique the use of "universal" manuals like the DSM and engage in creating culturally specific treatment that does not just pay "lip service" to the stereotypical associations made from our culture to another.

WORKS CITED

Kirmayer, Laurence J. "Beyond the ‘new cross-cultural psychiatry’: Cultural biology, discursive psychology and the ironies of globalization." *Transcultural psychiatry* 43.1 (2006): 126-144.

Ruth Benedict (1934) Anthropology and the Abnormal, The Journal of General Psychology, 10:1, 59-82, DOI: 10.1080/00221309.1934.9917714

Subandi, M. A., et al. "Sociocultural and Clinical Aspects of Recovery from First Episode Psychosis in Java, Indonesia: A Follow‐Up Case Study." *Ethos* 50.4 (2022): 410-433.

Wikan, Unni. "Managing the heart to brighten face and soul: Emotions in Balinese morality and health care." *American Ethnologist* 16.2 (1989): 294-312.